



تعميم رقم (129 / 2025) Circular No.

Date: 2025/07/24

التاريخ: 2025/07/24

To: All Licensed Pharmaceutical Facilities
in the Emirate of Abu Dhabi

إلى: جميع المنشآت الصيدلانية المرخصة في إمارة أبوظبي

Subject: Reminder - Compliance with
Patient Profiling as per DOH Standard
for Managing Supply and Safe Use of
Medications

الموضوع: تذكير بضرورة الالتزام بإعدادات ملف تعريف
المريض وفقاً لمعيار دائرة الصحة بشأن إدارة توريد الأدوية
والاستخدام الآمن لها

Greetings,

تحية طيبة وبعد ،،،

We extend our greetings and best wishes
for your continued success.

بدايةً، يسرنا أن نتقدم لكم بخالص التحية والتقدير
متمنين لكم دوام التوفيق والسداد.

With reference to the subject above, and to
ensure safe and effective use of
medications, we would like to remind all
pharmaceutical facilities licensed by the
Department of Health to comply with DOH
Standard no. (HAAD/MSSM/SD/1.0) for
Managing the Supply and Safe Use of
Medications where patients profiling is a
mandatory requirement.

بالإشارة إلى الموضوع أعلاه، ولضمان الاستخدام الآمن
والفعال للأدوية، فإننا نود تذكير جميع المنشآت
الصيدلانية المرخصة من دائرة الصحة بالامتثال لمعيار
دائرة الصحة رقم (HAAD/MSSM/SD/1.0) بخصوص
إدارة توريد الأدوية والاستخدام الآمن لها؛ حيث يُعتبر إعداد
ملف تعريف المريض متطلباً إلزامياً.

The details required in patients profiling
system have been previously explained
through awareness workshops.

وقد تم شرح التفاصيل المطلوبة في نظام إعدادات ملف تعريف
المريض سابقاً من خلال ورش عمل توعوية.

Thus, all pharmacies are deemed
accountable and are given a period of **three
months ONLY** from the date of this circular
to comply with this mandate.

وعليه، تُعتبر جميع الصيدليات مسؤولة بهذا الشأن، ويُمنح
لها مهلة **ثلاثة أشهر فقط** من تاريخ هذا التعميم للامتثال
لهذا المتطلب.

Please note that necessary disciplinary and
legal actions will be taken against
pharmaceutical facilities and professionals
found to be non-compliant.

وسيتخذ اتخاذ الإجراءات التأديبية والقانونية اللازمة في حق
المنشآت الصيدلانية والمهنيين المخالفين.

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لمزيد من الاستفسارات يرجى التواصل مع فريق التدقيق
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We hope that all will adhere to the above,
for the best interest of the health sector in
the Emirate.

أملين من الجميع الالتزام بما ورد أعلاه، لما فيه مصلحة
القطاع الصحي في الإمارة.

Thank you for your cooperation,

شاكرين لكم حسن تعاونكم معنا ،،،

"This circular is designed for regulatory procedures and
should not be used as content for media publication"

"هذا التعميم لإجراءات التنظيمية وغير مخصص كمحتوى للنشر الإعلامي".



بينة سيف العواني
عن/ د. نورة خميس الغيثي
وكيل دائرة الصحة





هيئة الصحة
HEALTH AUTHORITY

Document Title:	HAAD Standard for Managing The Supply and Safe Use of medications in licensed Healthcare Facilities		
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1. Purpose

- 1.1. This standard mandates the requirement to establish, manage and maintain a facility formulary system in healthcare facilities to ensure patient's access to an uninterrupted supply of safe, efficacious, and cost-effective medicines appropriate to their healthcare needs and consistent with the facility's scope of services.

2. Scope

- 2.1. This Standard applies to all healthcare facilities providing both in- and outpatient pharmacy services.
- 2.2. It covers the requirement to procure, manage safe dispensing and maintenance of adequate supply of necessary medicines and medicinal products in accordance with the facility's approved scope of services.

3. Definitions

- 3.1 **Drug:** a substance that is administered to treat, prevent or diagnose diseases that is used as a medication or in the preparation of a medication
- 3.2 **Devices that contain drugs:** A medical instrument with the sole purpose to administer a drug for treatment, screening or prevention purposes.
- 3.3 **Complementary Medicine Products:** Products that include drugs or devices to support treatment of patients through non-conventional medical methods.
- 3.4 **Generic Drugs:** term referring to the chemical makeup of a drug rather than to the advertised brand name under which the drug is sold.
- 3.5 **Returned and reused medication:** Medication that has been dispensed by the pharmacist (originator) for the purpose of treating a medical condition and by which has not been administered and therefore returned to the originator to assess its suitability and safety to be issued again for treatment purposes.
- 3.6 **Pharmacy and Therapeutics Committee (PTC):** is responsible for managing the formulary system, drug shortages, and managing the supply and safe use of medications in HAAD licensed healthcare facilities. It is composed of actively





practicing physicians, pharmacists, store managers, nurses, administrators, quality improvement managers, and other health care professionals and staff who participate in the medication-use process. Other responsibilities of the Pharmacy and Therapeutics committee may include but not limited to: Medication Use Evaluation (MUE), adverse-drug event monitoring and reporting, medication-error prevention, and support the development of clinical care plans and guidelines.

4. Duties for Healthcare Providers

- 4.1. Healthcare facilities and healthcare professionals must comply with the requirements of and duties as set out in this Standard and the respective HAAD Policies and Standards governing the use of medicines and medicinal products.
- 4.2. Report and submit data to HAAD via *e-claims* and as set out in the HAAD Data Standards and Procedures (found online at www.haad.ae/datadictionary).
- 4.3. Comply with HAAD policies and standards on managing patient medical records, including developing effective recording systems, maintaining patient records, maintaining confidentiality, privacy and security of patient information; and educating patients on services provided and medicinal products dispensed at the facility or administered by the professional and satisfying the requirements of patient rights and responsibilities charter.
- 4.4. Maintain and make available for HAAD audit, a list of the Facility Formulary, products dispensed (and administered) at the facility including drugs, drug containing devices, along with their prices.
- 4.5. Cooperate with the HAAD authorised auditors, during inspections and audits by HAAD.
- 4.6. Comply with the HAAD Standard for Narcotics and Controlled Medicinal Products.

5. Enforcement and Sanctions

- 5.1. HAAD licensed healthcare providers must comply with the requirements of this Standard, the HAAD Standard Provider Contract and the HAAD Data Standards and Procedures Standard. HAAD may impose sanctions in relation to any breach of requirements under this standard in accordance with Chapter IX, HAAD Policy on Complaints, Investigations, Regulatory Action, and Sanctions, The Healthcare Regulator Policy Manual Version 1.0.

6. Standard 1 – The Healthcare Facility Formulary System

- 6.1. Healthcare facilities with inpatient and outpatient must:
 - 6.1.1. establish, manage and maintain a Facility Formulary selected from HAAD Approved Drugs list
 - 6.1.1.1. is appropriate to the scope of services provided at the facility, and the patients' therapeutic needs; and
 - 6.1.1.2. is indexed by the generic name (active pharmaceutical ingredient).
 - 6.1.2. establish a Pharmacy and Therapeutics Committee (PTC) comprised of professionals with the appropriate skill mix and representation from facility professionals with the delegated authority to maintain a formulary and advise the facility management and clinicians on:
 - 6.1.2.1 the safe, evidence-based and cost – effective use of medicines and medicinal products;





- 6.1.2.2 supply and inventory (stocks held and patient needs) of facility specific medicines and medicinal products to serve the demand; and
- 6.1.2.3 developing and maintaining policies and guidelines concerning the selection, distribution and use of medicines and safety issues arising from their use.
- 6.1.3. ensure products are continuously available and adequate stocks are maintained for patients' treatment and medication needs without interruptions;
- 6.1.4. have in place procedures to manage drug shortage, and report such shortages to HAAD in accordance with relevant HAAD Standards;
- 6.1.5. ensure that healthcare professionals employed at their facility(s) comply:
 - 6.1.5.1. with any and all restriction(s) placed by HAAD or Federal Law on the use of certain medicines and medicinal products, including but not limited to narcotics and controlled medicinal products;
 - 6.1.5.2. with the specified scope of practice permitted by their license;
 - 6.1.5.3. with the requirements for specialised training and continuing professional development associated with prescribing and medication management, as specified by HAAD Healthcare Providers Policy Manual.

7. Standard 2. Healthcare Facility Inventory and Product Storage Management

- 7.1. Procurement of medicines and medicinal products
 - 7.1.1. Healthcare facilities must:
 - 7.1.1.1. ensure that medicines and medicinal products are procured from appropriately registered/authorised suppliers/distributors and avoid substandard and counterfeit drugs;
 - 7.1.1.2. have a process in place to notify healthcare professionals responsible for prescribing medicines and medicinal products when medicines on the Facility Formulary are out-of-stock and to recommend substitutions ensuring an uninterrupted availability/supply;
 - 7.1.1.3. report shortages of medicines and medicinal products to HAAD, in accordance with relevant HAAD Standards.
- 7.2. Recall of medicines and medicinal products
 - 7.2.1. Healthcare facilities must have a recall system in place, including an identified responsible and authorised officer to manage the recall, whereby medicinal products subject to HAAD initiated recall can be:
 - 7.2.1.1. immediately identified;
 - 7.2.1.2. removed from the active inventory at the shortest time period, not exceeding 24 hours from the time of receipt of recall;
 - 7.2.1.3. confiscated;
 - 7.2.1.4. notified to all relevant healthcare professionals; and
 - 7.2.1.5. notified to all relevant patients.
 - 7.2.2 A record of actions taken must be documented and maintained along with the recall notice (including none, if found in inventory and the date the action was taken and/or closed). The record must also include:
 - 7.2.2.1 length of time from receipt of the recall notice until closure is attained;
 - 7.2.2.2 the location and quantity of recalled product returned; and
 - 7.2.2.3 identification of any problems encountered with the recall process.





7.3. Storage of medicines and medicinal products

7.3.1. Healthcare facilities must provide adequate storage area for medicines and medicinal products and must ensure that all such products are stored and transported in a clean and safe environment under conditions suitable for product stability and safety, including but not limited to for:

7.3.1.1. products requiring cold chain management such as biologicals and vaccines, in accordance with the HAAD Standard for Safe Vaccine Handling and Cold Chain maintenance;

7.3.1.2. all blood products, in accordance with the manufacturer's recommendations;

7.3.2. Healthcare facilities must ensure that the facility's pharmacists-in-charge:

7.3.2.1. Make sure that the refrigerator temperature is within the limit (2-8C) and check the temperature log sheet on daily basis.

7.3.2.2. A backup plan must be in place mitigate faulty refrigeration.

7.3.2.3. inspects the medicines and medicinal products storage areas at least once a month;

7.3.2.4. document the findings in a report, which must be signed by the pharmacist-in-charge and the responsible senior manager of the facility;

7.3.2.5. identified issues and improvement actions must be implemented, and evidence must be retained on facility(s) records, and made available to HAAD authorised inspectors when requested.

7.4. Inventory Control of medicines and medicinal products

7.4.1 Healthcare facilities must establish and manage a medication inventory in a systematic manner assigning responsibility for inventory control to identified healthcare professional(s), including for

7.4.1.1 checking for expired medicines and medicinal products at least once a month and remove expired medication from the active medication inventory and dispose of expired products in accordance with HAAD EHSMS Standard for Health Care Providers Waste Management;

7.5 Reuse of dispensed products in inpatient settings

7.5.1 Reuse dispensed unused medications for another inpatient is permitted under the following conditions and specifications for the dispensed medication:

7.5.1.1 was issued to an inpatient admitted to the facility concerned;

7.5.1.2 returned to the pharmacy within 3 working days from issuance;

7.5.1.3 has an expiration date of at least three months from the date of return;

7.5.1.4 does not require refrigeration or not known to be oxygen sensitive or does not exhibit moisture or light sensitivity;

7.5.1.5 was repackaged in a safely sealable pack/blister for single unit use and demonstrates integrity and stability;

7.5.1.5 was labeled with the product name, dose, date of issue, batch number/lot number and expiry date;

7.5.1.6 was returned to the pharmacist in-charge at the concerned facility;

7.5.1.7 is recorded on a register as returned with the details of





- patient, condition, issuance date, dose, prescribing physician and expiry date of at least six months. Records must be maintained for HAAD audits; and
- 7.5.1.8 is stored under appropriate conditions and independent of the original inventory from which it was dispensed;
- 7.5.1.9 Is not listed as a Narcotic or Controlled Medicinal Product.
- 7.6 The Healthcare Facility must ensure:
 - 7.6.1 it does not return medication previously dispensed for the use by an out-patient or a discharged in-patient to the inventory or for reuse by another patient;
 - 7.6.2 apply appropriate measures to keep the medication inventory secure;
 - 7.6.3 identify, record and maintain reports on unsafe practices related to medication inventory and records management; and
 - 7.6.4 Ensure evaluation is undertaken by appropriately qualified pharmacy staff to ensure such medications are not adulterated or misbranded.

8 Standard 3. Medication Labeling Standards

- 8.1 Healthcare facilities must comply with the following HAAD labeling requirements for medicines and medicinal products:
 - 8.1.1 Inpatient pharmacies:
 - 8.1.1.1 all medication prepared by the pharmacy for future use in the facility, which is not intended for immediate dispensing to a patient must be identified as follows:-
 - 8.1.1.1.1 single dose or multi-dose drugs, except parenteral solutions in which a drug has been added must be labeled with:
 - 8.1.1.1.1.1 Drug name;
 - 8.1.1.1.1.2 Strength;
 - 8.1.1.1.1.3 Amount and if applicable dose;
 - 8.1.1.1.1.4 Expiry date;
 - 8.1.1.1.1.5 Lot number or reference code.
 - 8.1.1.1.2 parenteral solutions, which have drugs added must contain the following information on the outer label:
 - 8.1.1.1.2.1 Name of each drug, strength and amount including base parenteral solution;
 - 8.1.1.1.2.2 Expiry date and time of the admixture;
 - 8.1.1.1.2.3 Lot number or reference code.
 - 8.1.1.2 medication prepared by the pharmacy for immediate dispensing to a specific patient in the facility must be identified with the following information:
 - 8.1.1.2.1 single or multi-dose drugs, except parenteral solutions to which a drug has been added must be identified with:
 - 8.1.1.2.1.1 Drug name;
 - 8.1.1.2.1.2 Strength, and amount/dose, as applicable.
 - 8.1.1.2.2 parenteral solutions which have drugs added must be identified with:





- 8.1.1.2.2.1 Name, concentration and volume of the base parenteral solution;
 - 8.1.1.2.2.2 Expiry date and time.
 - 8.1.1.2.2.3 Lot number or reference code.
 - 8.1.1.3 medications dispensed to a specific patient in the facility must be dispensed in a container identified with the patient name and location;
 - 8.1.1.4 the dispensing pharmacist must dispense medications in the most ready to administer form in the in-patient environment.
- 8.1.2 Outpatient pharmacies
 - 8.1.2.1 all medication prepared by the pharmacy for immediate dispensing to a patient must be identified with the following information:
 - 8.1.2.1.1 Patient name;
 - 8.1.2.1.2 Date of prescription fill;
 - 8.1.2.1.3 Facility name and address;
 - 8.1.2.1.4 Name of the prescriber;
 - 8.1.2.1.5 Drug name;
 - 8.1.2.1.6 Strength and dosage form;
 - 8.1.2.1.7 Quantity dispensed;
 - 8.1.2.1.8 Route of administration;
 - 8.1.2.1.9 Directions for use and cautionary statements (where applicable);
 - 8.1.2.1.10 Expiry date of the medication;
 - 8.1.2.1.11 Name, initials or unique identifier of the pharmacist that filled or refilled the prescription.
 - 8.1.2.2 Labelling of all dispensed medicines must be clear, legible and indelible; and
 - 8.1.2.3 Labels for compounded sterile products must not limit visual inspection of the container contents, where possible.

9 Standard 4. Medication Process

9.1 Prescribing and ordering medicines and medicinal products

9.1.1 Healthcare facilities must ensure that:

- 9.1.1.1 medications are prescribed only by healthcare professionals given prescribing authority by law and HAAD licensure, scopes of practice, certification and applicable HAAD Policies and Standards;
 - 9.1.1.2 verbal medication orders, where allowed, must be supported by facility policies and standard operating procedures and ensure that the process is regularly monitored and evaluated to ensure adequate recording of verbal orders and assuring patient quality and safety;
- 9.1.2 Healthcare professionals authorised to prescribe medication in accordance to section 9 must comply with:
 - 9.1.2.1 the HAAD approved prescribing process, including where specified by HAAD the mechanism/process of transmission of prescription, its format and specified forms, the coding and data





entry specifications and the number of repeats and/or refills allowed for a particular medication;

10 Standard 5. Prescribing of medication and refills

10.1 When prescribing medication and repeat refills are determined to be required for the patient, the maximum number of refills allowed and that must:

- 10.1.1 Not exceed 12 months' supply from the date of initiating a prescription
 - 10.1.1.1 Exceptions may be allowed for longer duration without seeing a physician if the patient has mitigating factors (e.g. seen by a physician outside of Abu Dhabi)
- 10.1.2 Ensure that a refill provides a supply of up to 3 months, if so necessitated by the patient's condition;
- 10.1.3 Ensure that no more than total of 3 repeat refills is prescribed with each refill for up to 3 months, where necessitated by the patient's condition
- 10.1.4 Validity of the refill will be one month from the designated date of the next refill
- 10.1.5 Early refill shall be accepted but no more than 10 days from the date of next refill
- 10.1.6 Repeat/refill prescriptions must only be issued where evidence exists of a chronic condition, the patient's condition is considered stable by the treating physician and the physician has a management care plan for the patient, and has documented evidence to this effect in support of the case concerned;
- 10.1.7 Medications used to treat acute conditions are excluded from refill dispensing such as antibiotics, over the counter medications, infertility, and sexual dysfunction, etc.
- 10.1.8 Coding must be in accordance with HAAD Data Standards and Procedures and HAAD Data Dictionary found at www.haad.ae/DataDictionary;

10.2 Any restrictions and/or controls specified by HAAD for prescribing, ordering and transcribing orders are met, in accordance with requirements as mandated in the HAAD Approved Products list and Standards for the Management of Narcotics and Controlled Medicinal Products and the HAAD Standard for Generic Drugs;

10.3 Healthcare professionals must not sell, dispense or distribute drug samples to patients.

10.4 Preparation of medicines:

- 10.4.1 Healthcare facilities must ensure that where certain medications are repackaged for use by the facility, including for medications intended for unit-of-use dosing and smaller packs appropriate for prescribed period, that the re-packaging of medications in single units, and their relabeling complies with the requirements set out in this standard (refer to section 8);
- 10.4.2 where compounding of pharmaceuticals is performed at the facility, that the process is:
 - 10.4.2.1 supported by facility policies and standard operating procedures;





- 10.4.2.2 performed under the supervision and review of a HAAD licensed pharmacist, with the requisite competence, experience and skills to do so;
- 10.4.2.3 done in a clean and safe area, and with appropriate equipment and supplies;
- 10.4.2.4 sterile admixture services must adhere to strict aseptic techniques;
- 10.4.2.5 cytotoxic drugs must only be handled and prepared by appropriately trained pharmacy personnel under appropriate occupational health and safety controls in accordance with the respective requirements of the HAAD EHSMS Standards; compounded products are tested and assessed for safety, contamination and content;
- 10.4.2.5 documentation and records are kept and maintained on all compounded products.

11. Standard 6. Dispensing Medication, Patient Education, and Record Keeping

11.1 Healthcare facilities must:

- 11.1.1 ensure that healthcare professionals employed at their facilities comply with the HAAD requirements for dispensing medicines and medicinal products, including the following:
 - 11.1.2.1 The pharmacist must be licensed by HAAD, and must personally dispense the prescribed medication;
 - 11.2.2 the technician may prepare medication but this must be checked and dispensed by a HAAD licensed pharmacist;
 - 11.1.2.3 All Prescription records for non- controlled medicinal products are maintained in a readily retrievable format in a secure area for a total of 3 years from the date of the last transaction; and
 - 11.1.2.4 Data, if in computerised prescription storage systems must facilitate backup and can be retrieved in case of need.

11.2 The pharmacist must:

- 11.2.1 not dispense any medicine that is Prescription-Only-Medicine (POM) without a formal prescription that complies with the HAAD approved format;
- 11.2.2 must review each prescription and order for prior to dispensing a medication for its:
 - 11.2.2.1 Authenticity and accuracy;
 - 11.2.2.2 validity of prescription (not more than 2 days for narcotic and controlled medicines, not more than 3 days for semi-controlled Medicines and 60 days for uncontrolled medicines from the date of issue of prescription);
 - 11.2.2.3 appropriateness of the medication;
 - 11.2.2.4 any contraindication noted;
 - 11.2.2.5 the appropriateness of the dose and the route of administration;





- 11.2.2.6 the dispensed medication(s) complies with HAAD labeling standards (section 8);
- 11.2.2.7 complies with the requirements of the HAAD Standard for Generic Drugs including the restrictions imposed on therapeutic substitution of a prescribed medication; and
- 11.2.2.8 sufficient and relevant information is provided to the patient to ensure safe and effective use of medications;
- 11.2.3 where necessary, the pharmacist must contact the prescribing physician to clarify any information on the prescription and document the time and date of the contact.
- 11.2.4 the pharmacist may refuse to fill a prescription/order that, in his/her professional judgment, is unsafe or outside the bounds of safe and effective medication therapy. Where this may be the case, the pharmacist must communicate with the prescribing physician to address the issue and to ensure the patient has timely access to appropriate medication. The Pharmacist must not change the patient's prescription without the treating physician's approval;
- 11.2.5 the pharmacist must only refill a prescription in accordance with the specifications and number of repeats specified by the prescribing physician, and in accordance with HAAD requirements as per section 9.1.

12 Standard 7. Administering and Monitoring Medicines

- 12.1 Healthcare facilities, and pharmaceutical facilities, must:
 - 12.1.1 ensure that medications are administered only by individuals authorised by HAAD licence, privileging and HAAD Policies and Standards to administer medicines and medicinal products;
 - 12.1.1 have in place policies and procedures to ensure that medications are dispensed and administered accurately, to the intended patient, at the intended time, by appropriate and duly authorised (by licence and scope) personnel and as prescribed/ordered by the physician;
 - 12.1.2 ensure that medications administered must be documented in the patient's medical record, including details of daily, weekly, monthly or otherwise, as well as times of scheduled medications, where appropriate;
 - 12.1.2.1 ensure that medication errors and adverse drugs reactions are recorded and reported in accordance with the HAAD Adverse drug reaction & medication error policies.

13 Standard 8. Emergency Medications

- 13.1 Healthcare facilities must comply with the requirements of the HAAD Standard for Minimum Preparedness for Common Medical Emergencies as applicable to the setting.





14 Standard 9. Requirements for Controlled Drugs

- 14.1 In addition to the requirements set forth in this Standard, healthcare facilities, pharmaceutical facilities and professionals must comply with the requirements of the HAAD Standard for the Management of Narcotics and Controlled Medicinal Products.

15 Standard 10. Requirements for Radiopharmaceuticals

15.1 Healthcare facilities must:

- 15.1.1 purchase radiopharmaceuticals only from authorised suppliers and must ensure that these are used within the declared shelf-life;
- 15.1.2 record the contents and the result of inspection for physical damage and measurement of radiation dose-rate of the product on receipt;
- 15.1.3 maintain and store records and receipts for radioactive material, including identification of the radiopharmaceutical, its source, the amount of activity received, and the results of radiation surveys and contamination testing. Any discrepancies must be reported to the manufacturer/supplier;
- 15.1.4 ensure that all radiopharmaceuticals dispensed and administered must be preceded by a prescription from an appropriately licensed and duly authorised (by license or privilege) prescriber; all such prescriptions must specify the procedure desired, the drugs to be used, the amounts to be administered, the route of administration and, if applicable, the rate of infusion;
- 15.1.5 standing orders for defined radiopharmaceuticals and amount may be created for each procedure (defined either by protocol or a written individual medication order). Any variation to a protocol (e.g., patient does not meet criteria or physician chooses to modify a protocol) must be approved by a physician with clinical privileges to do so;
- 15.1.6 ensure that disposal of radiopharmaceuticals is done in accordance with the HAAD EHSMS respective Standards;





- 15.2 healthcare professionals prescribing, dispensing and administering radiopharmaceuticals must:
 - 15.2.1 follow HAAD Standards, where available, or international evidence based care standards;
 - 15.2.2 document patient history and medication to ensure patient safety when administering radiopharmaceuticals;
 - 15.2.3 verify the identity of the radiopharmaceutical and patient and the route of administration before administration;
 - 15.2.4 ensure that a pregnancy test must be done on all female patients before administration of radio pharmaceuticals.

16 Standard 11. Requirements for Investigational Medicinal Products

- 16.1 Research involving humans
 - 16.1.1 Healthcare facilities and professionals engaged in human subject research including the use of investigational medicinal products must comply with the requirements of the HAAD policies and standards governing human subject research, Chapter V Human Subject Research, Healthcare Regulator Policy Manual Version 1.0.
- 16.2 Inventory management
 - 16.2.1 Healthcare facilities must ensure that:
 - 16.2.1.1 The pharmacy has a separate storage area for investigational medicinal products;
 - 16.2.1.2 investigational medicinal products are labeled with the message “For Investigational Medicinal Use Only” to distinguish them from other medicines and medicinal products labels;
 - 16.2.1.3 Inventory of investigational medicinal products is performed at least monthly or more often, if indicated in the study protocol. Investigational medicinal products accountability records must be retained by the pharmacy responsible and duly authorised professional for a period of time as required by the study protocol;
 - 16.2.1.4 At the conclusion of the study, the pharmacy responsible and duly authorised professional reconciles all documented used and unused doses and returns all unused or contaminated investigational medicinal products to the principal investigator or the study sponsor as indicated in the study protocol.





16.3 Dispensing investigational drugs

16.3.1 Healthcare professionals involved in an investigational study, and principal investigators, must ensure that:

- 16.3.1.1 A signed, informed consent form is verified before an investigational medicinal product is dispensed;
- 16.3.1.2 investigational medicinal products are only dispensed on the receipt of a signed written or electronically duly authorized (for research purposes) prescription from an appropriately licensed and duly authorised (for research) investigator;
- 16.3.1.3 Records of all dispensed investigational medicinal products stocked in the pharmacy are maintained. Records must be kept for 2 years after an application for approval for investigational use.

16.4 Administration of investigational drugs

16.4.1 Healthcare professionals involved in an investigational study, and principal investigators, must ensure that:

- 16.4.1.1 investigational medicinal products are administered only under the direct supervision of the Principal Investigator and by a licensed and appropriately privileged healthcare professional with the requisite competence and skills to administer the investigational drug in accordance with the study protocol;
- 16.4.1.2 when nurses are called upon to administer investigational medicinal product, that this is done under direct supervision as per section 12, and that they are adequately informed about the nature of the medication, the dosage forms, the strengths available, actions and therapeutic indications, side effects and symptoms of toxicity.

